

Sample questions about quality

1-MCQ:

1. What is the definition of quality in healthcare?

- a. Doing many procedures quickly
- B. Doing the right thing at the right time in the right way
- C. Treating only serious diseases
- D. Reducing hospital staff

Answer: B

2. According to the WHO, quality of care improves:

- a. Hospital buildings
- B. Doctors' salaries
- C. Desired health outcomes
- D. Medical equipment only

Answer: C

3. Which of the following is a dimension of quality in healthcare?

- a. Profitability
- B. Safety
- C. Advertising
- D. Marketing

Answer: B

4. Delivering healthcare that minimizes risks and harm refers to:

- a. Efficiency
- B. Safety
- C. Timeliness
- D. Equity

Answer: B

5. Providing evidence-based care that achieves desired outcomes is called:

- a. Efficiency
- B. Effectiveness
- C. Equity
- D. Safety

Answer: B

6. Optimizing resource use and avoiding waste is known as:

- a. Efficiency
- B. Safety
- C. Timeliness
- D. Patient satisfaction

Answer: A

7. Delivering care promptly and reducing waiting times refers to:

- a. Equity
- B. Timeliness
- C. Safety
- D. Effectiveness

Answer: B

8. Healthcare that does not differ in quality due to gender, race, or socioeconomic status is called:

- a. Efficient care
- B. Equitable care
- C. Timed care
- D. Evidence care

Answer: B

9. Care that focuses on patient needs, values, and preferences is:

- a. Patient-centered care
- B. Financial care
- C. Research care
- D. Administrative care

Answer: A

10. Safety in healthcare mainly focuses on:

- a. Increasing patient numbers
- B. Preventing harm to patients
- C. Increasing hospital profits
- D. Hiring more doctors

Answer: B

11. Minimizing errors, adverse events, and injuries is part of:

- a. Preventing harm
- B. Marketing
- C. Budgeting
- D. Scheduling

Answer: A

12. Identifying and addressing potential hazards is called:

- a. Efficiency
- B. Risk mitigation
- C. Patient loyalty
- D. Diagnosis

Answer: B

13. Encouraging transparency and continuous improvement is part of:

- a. Safety culture
- B. Hospital management
- C. Patient billing
- D. Marketing strategy

Answer: A

14. High-quality and safe healthcare helps improve:

- a. Patient outcomes
- B. Hospital parking
- C. Medical advertisements
- D. Staff uniforms

Answer: A

15. Quality care increases patient:

- a. Fear
- B. Dissatisfaction
- C. Satisfaction and trust
- D. Waiting time

Answer: C

16. Preventing errors and complications helps:

- a. Increase costs
- B. Reduce healthcare costs
- C. Reduce staff
- D. Increase hospital buildings

Answer: B

17. A good reputation for healthcare organizations is achieved by:

- a. Advertising
- B. Delivering high-quality safe care
- C. Increasing fees
- D. Hiring fewer staff

Answer: B

18. Meeting quality and safety standards ensures:

- a. Profit increase
- B. Regulatory compliance
- C. Less work
- D. Faster discharge

Answer: B

19. Care that uses the best available research evidence is called:

- a. Traditional care
- B. Evidence-based practice
- C. Experimental care
- D. Random care

Answer: B

20. A culture that balances accountability with learning from errors is called:

- a. Safety culture
- B. Just culture
- C. Economic culture
- D. Administrative culture

Answer: B

Quality improvement

21. Quality improvement in healthcare is a framework used to:

- a. Increase hospital buildings
- B. Systematically improve how care is delivered
- C. Reduce the number of patients
- D. Increase advertising

Answer: B

22. Which of the following is a characteristic of the quality improvement process?

- a. Ignored
- B. Measured
- C. Hidden
- D. Random

Answer: B

23. The quality improvement process involves:

- a. Measured, analyzed, controlled, and improved
- B. Treated, discharged, and billed

- C. Diagnosed and ignored
- D. Advertised and promoted

Answer: A

24. Which organization provides international healthcare accreditation standards?

- a. WHO
- B. JCI
- C. UNICEF
- D. FDA

Answer: B

25. The quality initiatives of CMS emphasize:

- a. Marketing and profit
- B. Quality, safety, and value-based care
- C. Buildings and equipment
- D. Patient numbers

Answer: B

26. The Agency for Healthcare Research and Quality (AHRQ) focuses on:

- a. Safety and evidence-based practice
- B. Hospital construction
- C. Staff recruitment
- D. Advertising

Answer: A

27. Which organization focuses on quality, safety, and continuous improvement?

- a. IHI
- B. WHO
- C. Red Cross
- D. UNICEF

Answer: A

28. Achieving CBAHI accreditation is mandatory for healthcare facilities in:

- a. Egypt
- B. Saudi Arabia
- C. USA
- D. Canada

Answer: B

29. Which organization leads healthcare accreditation and regulation in Egypt?

- a. JCI
- B. CMS
- C. GAHAR
- D. WHO

Answer: C

30. Quality and safety in healthcare are:

- a. Separate concepts
- B. Opposite ideas
- C. Interdependent and interconnected
- D. Unrelated

Answer: C

31. High-quality care reduces:

- a. Staff salaries
- B. Errors and adverse events
- C. Hospital buildings
- D. Medical equipment

Answer: B

32. A safety-focused culture helps:

- a. Reduce transparency
- B. Improve quality
- C. Decrease patient care
- D. Stop improvement

Answer: B

33. Quality and safety share common goals such as:

- a. Improving patient outcomes
- B. Increasing waiting times
- C. Increasing hospital costs
- D. Reducing staff

Answer: A

34. Conducting regular quality and safety assessments helps:

- a. Identify areas for improvement
- B. Increase patient waiting time
- C. Reduce staff training
- D. Decrease safety

Answer: A

35. Implementing evidence-based practices means:

- a. Using traditional habits only
- B. Using the best available research evidence
- C. Ignoring research
- D. Random decision making

Answer: B

36. Encouraging reporting and transparency is part of:

- a. Financial culture
- B. Safety culture
- C. Political culture
- D. Social culture

Answer: B

37. Engaging patients and families in healthcare decisions is known as:

- a. Administrative approach
- B. Multidisciplinary approach
- C. Multiciliary approach
- D. Financial approach

Answer: C

38. Providing ongoing education and training helps:

- a. Reduce staff knowledge
- B. Improve staff skills
- C. Increase errors
- D. Delay treatment

Answer: B

39. One challenge facing healthcare quality and safety is:

- a. Too few patients
- B. Complexity of healthcare systems
- C. Too many hospitals
- D. Too much free time

Answer: B

40. Value-based care focuses on:

- a. Increasing costs
- B. Quality, safety, and cost-effectiveness
- C. Advertising hospitals

D. Reducing healthcare services

Answer: B

Risk Management and Error Reduction in Healthcare

41. What does IPSGs stand for?

- a. International Patient Safety Goals
- B. International Patient Service Group
- C. Internal Patient Safety Guide
- D. International Public Safety Group

Answer: A

42. Correct patient identification is part of:

- a. Hospital management rules
- B. International Patient Safety Goals
- C. Medical research
- D. Hospital marketing

Answer: B

43. Improving staff communication is important for:

- a. Patient safety
- B. Hospital decoration
- C. Marketing
- D. Finance

Answer: A

44. In healthcare, risk refers to:

- a. Guaranteed harm
- B. Potential for harm or injury
- C. Patient satisfaction
- D. Hospital cost

Answer: B

45. An error in healthcare is defined as:

- a. Planned medical action
- B. Unintended act or omission
- C. Financial loss
- D. Hospital delay

Answer: B

46. An adverse event is:

- a. Intentional patient harm
- B. Harm caused by natural disease only
- C. Unintended harm resulting from medical care
- D. Planned surgical outcome

Answer: C

47. A near miss is:

- a. Harm that already occurred
- B. An event that could have caused harm but was prevented
- C. A patient complaint
- D. A medical diagnosis

Answer: B

48. Giving the wrong medication or wrong dose is an example of:

- a. Diagnostic error
- B. Medication error
- C. Equipment failure
- D. Infection

Answer: B

49. Infections acquired during a hospital stay are called:

- a. Community infections
- B. Seasonal infections
- b. Viral infections
- D. Hospital-acquired infections (HAIs)

Answer: D

50. Patient slips, trips, or falls in hospitals are examples of:

- a. Medication errors
- B. Falls and injuries
- C. Equipment failure
- D. Diagnostic errors

Answer: B

51. Infections that occur at the site of surgery are called:

- a. Surgical Site Infections (SSIs)
- B. Skin infections
- C. Viral infections
- D. Respiratory infections

Answer: A

52. Venous Thromboembolism (VTE) refers to:

- a. Skin disease
- B. Blood clots in veins
- C. Bone fractures
- D. Muscle pain

Answer: B

53. Skin damage caused by prolonged pressure is called:

- a. Skin allergy
- B. Pressure ulcer
- C. Infection
- D. Burn

Answer: B

54. Incorrect or delayed diagnosis is known as:

- a. Medication error
- B. Diagnostic error
- C. Communication error
- D. Surgical error

Answer: B

55. Poor communication between healthcare staff may lead to:

- a. Improved care
- B. Communication breakdown
- C. Faster diagnosis
- D. Better treatment

Answer: B

56. Staff fatigue and stress are examples of:

- a. Environmental factors
- B. Human factors
- C. Technological failures
- D. Financial issues

Answer: B

57. Equipment malfunction is considered:

- a. Human factor
- B. Environmental factor
- C. Technological failure
- D. Communication error

Answer: C

58. One consequence of patient safety risks is:

- a. Improved hospital reputation
- B. Patient harm or injury
- C. Lower healthcare costs
- D. Increased patient satisfaction

Answer: B

59. Management is defined as:

- a. Ignoring healthcare problems
- B. Coordinating tasks using available resources to achieve goals
- C. Reducing staff responsibilities
- D. Increasing hospital advertising

Answer: B

60. One benefit of effective risk management is:

- a. Increased errors
- B. Improved patient safety
- C. Reduced staff training
- D. More medical mistakes

Answer: B

Levels of Management, Risk Management, and Patient Safety

61. The term Levels of Management refers to:

- a. Hospital departments
- B. A line of demarcation between managerial positions
- C. Number of workers
- D. Medical procedures

Answer: B

62. The levels of management are divided into:

- a. Two levels
- B. Three levels
- C. Four levels
- D. Five levels

Answer: B

63. Which of the following is a level of management?

- a. Administrative level
- B. Laboratory level
- C. Pharmacy level
- D. Patient level

Answer: A

64. Top management mainly focuses on:

- a. Cleaning hospital rooms
- B. Planning and policy making
- C. Giving injections
- D. Laboratory testing

Answer: B

65. Top management includes:

- a. Nurses
- B. Supervisors
- C. Board of directors
- D. Workers

Answer: C

66. Top management is responsible for:

- a. Cleaning wards
- B. Preparing strategic plans
- C. Patient admission
- D. Laboratory tests

Answer: B

67. Who appoints middle-level managers?

- a. Workers
- B. Top management
- C. Patients
- D. Supervisors

Answer: B

68. Middle-level management includes:

- a. Workers
- B. Branch managers and departmental managers
- C. Patients
- D. Pharmacists only

Answer: B

69. Middle managers are responsible to:

- a. Workers
- B. Top management

- C. Patients
- D. Government

Answer: B

70. Middle managers mainly:

- a. Execute organizational plans
- B. Clean hospital rooms
- C. Admit patients
- D. Provide medications

Answer: A

71. Lower level management is also called:

- a. Administrative level
- B. Supervisory level
- C. Financial level
- D. Strategic level

Answer: B

72. Lower-level managers include:

- a. Supervisors and foremen
- B. CEOs
- C. Board members
- D. Government officials

Answer: A

73. Lower-level managers mainly:

- a. Make national policies
- B. Supervise workers
- C. Design hospitals
- D. Manage finances

Answer: B

74. Assigning tasks to workers is the responsibility of:

- a. Top management
- B. Middle management
- C. Lower-level management
- D. Government

Answer: C

75. Managers in risk management should have:

- a. Budgeting skills
- B. Cooking skills
- C. Driving skills
- D. Farming skills

Answer: A

76. Which skill is important for managers?

- a. Communication skills
- B. Singing skills
- C. Acting skills
- D. Drawing skills

Answer: A

77. Risk in healthcare means:

- a. Guaranteed harm
- B. Potential for harm
- C. Patient satisfaction
- D. Hospital income

Answer: B

78. Error in healthcare is:

- a. Intentional act
- B. Unintended act or omission
- C. Planned treatment
- D. Hospital policy

Answer: B

79. An adverse event is:

- a. Planned procedure
- B. Harm resulting from medical care
- C. Hospital advertisement
- D. Normal recovery

Answer: B

80. A near miss is:

- a. Harm that occurred
- B. Event that could cause harm but was prevented
- C. Patient complaint
- D. Medical diagnosis

Answer: B

81. Identifying patients correctly is part of:

- a. Hospital marketing
- B. International Patient Safety Goals
- C. Laboratory testing
- D. Financial management

Answer: B

82. Improving staff communication helps:

- a. Increase errors
- B. Improve patient safety
- C. Reduce hospital size
- D. Increase waiting time

Answer: B

83. Medication errors include:

- a. Correct drug administration
- B. Wrong drug or wrong dose
- C. Proper diagnosis
- D. Patient education

Answer: B

84. Infections acquired during hospital stay are called:

- a. Viral infections
- B. Hospital-acquired infections
- C. Skin infections
- D. Seasonal infections

Answer: B

85. Patient falls are considered:

- a. Safety risks
- B. Marketing strategies
- C. Financial issues
- D. Administrative tasks

Answer: A

86. Blood clots in veins are called:

- a. VTE
- B. HAI
- C. SSI
- D. RCA

Answer: A

87. Pressure ulcers occur due to:

- a. Infection only
- B. Prolonged pressure on skin
- C. Medication errors
- D. Surgery

Answer: B

88. Equipment malfunction is an example of:

- a. Human factor
- B. Technological failure
- C. Communication breakdown
- D. Environmental factor

Answer: B

89. Staff fatigue is considered:

- a. Human factor
- B. Environmental factor

- C. Financial factor
- D. Administrative factor

Answer: A

90. One consequence of patient safety risks is:

- a. Improved health
- B. Patient harm
- C. Reduced hospital work
- D. Increased satisfaction

Answer: B

91. Failure Mode and Effects

Analysis (FMEA) is used to:

- a. Identify potential failures
- B. Diagnose diseases
- C. Treat patients
- D. Discharge patients

Answer: A

92. Root Cause Analysis (RCA) is used to:

- a. Identify underlying causes of adverse events
- B. Increase hospital revenue
- C. Train patients
- D. Discharge patients

Answer: A

93. SWOT analysis evaluates:

- a. Safety, Work, Order, Treatment
- B. Strengths, Weaknesses, Opportunities, Threats
- C. Staff, Workers, Operations, Training
- D. Strategy, Work, Organization, Technology

Answer: B

94. Fishbone diagrams are used to:

- a. Identify causes of problems
- B. Diagnose patients
- C. Measure blood pressure
- D. Train nurses

Answer: A

95. Pareto analysis identifies:

- a. Rare problems
- B. Most common problems
- C. Patient satisfaction
- D. Staff salaries

Answer: B

96. Risk matrix helps to:

- a. Diagnose diseases
- B. Prioritize risks
- C. Admit patients
- D. Increase hospital size

Answer: B

97. Barcode scanning systems help reduce:

- a. Medication errors
- B. Patient falls
- C. Infections
- D. Equipment failure

Answer: A

98. Fall risk assessments help prevent:

- a. Medication errors
- B. Patient falls
- C. Infections
- D. Equipment malfunction

Answer: B

99. Regular inspection of equipment helps prevent:

- a. Surgical errors
- B. Equipment malfunction
- C. Patient falls
- D. Communication errors

Answer: B

100. One challenge in risk management is:

- a. Too many hospitals
- B. Limited resources
- C. Too many nurses
- D. Too few patients

Answer: B

Open Questions with Answers

101. Define quality in healthcare.

Answer:

Quality in healthcare means **doing the right thing at the right time in the right way to achieve the best possible results for patients.**

102. According to WHO, what is quality of care?

Answer:

Quality of care is **the extent to which healthcare services provided to individuals and populations improve desired health outcomes and are safe, effective, timely, efficient, equitable, and patient-centered.**

103. List the six dimensions of quality in healthcare.

Answer:

Safe
Effective
Efficient
Timely
Equitable
Patient-centered

104. Define the following terms: risk, error, adverse event, and near miss.

Answer:

Risk: Potential for harm or injury to patients, staff, or visitors.

Error: Unintended act or omission that may lead to harm.

Adverse event: Unintended harm resulting from medical care.

Near miss: An event that could have caused harm but was prevented.

105. List five common patient safety risks in healthcare settings.

Answer:

Medication errors
Hospital-acquired infections (HAIs)
Patient falls and injuries
Surgical site infections (SSIs)
Diagnostic errors
(Other examples: pressure ulcers, VTE, equipment failure, communication breakdowns)

106. What are the International Patient Safety Goals (IPSGs)?

Answer:

- Identify patients correctly
- Improve staff communication
- Use medicines safely
- Use alarms safely
- Prevent infections
- Identify patient safety risks
- Improve healthcare quality
- Prevent surgical mistakes

107. Explain the three levels of management.

Answer:

Top-level management: Includes board of directors and executives; responsible for policies, planning, and organizational goals.

Middle-level management: Includes departmental and branch managers; responsible for implementing policies and coordinating departments.

Lower-level management: Includes supervisors and foremen; responsible for supervising workers and managing daily activities.

108. What are the main roles of lower-level managers?

Answer:

- Assign tasks to workers
- Guide and supervise workers
- Maintain quality and quantity of work
- Maintain good relationships in the organization
- Provide training and motivation

109. List four risk management tools used in healthcare.

Answer:

- Failure Mode and Effects Analysis (FMEA)
- Root Cause Analysis (RCA)

SWOT Analysis

Fishbone Diagram

110. Mention five strategies for improving quality and safety in healthcare.

Answer:

- Conduct regular quality and safety assessments
- Implement evidence-based practices
- Foster a safety culture
- Engage patients and families in care decisions
- Provide continuous education and training for staff

111. List the characteristics of the quality improvement process.

Answer:

- Measured
- Analyzed
- Controlled
- Improved

112. List four accreditation initiatives that integrate quality and safety standards.

Answer:

- JCI (Joint Commission International)
- CMS (Centers for Medicare and Medicaid Services)
- AHRQ (Agency for Healthcare Research and Quality)
- IHI (Institute for Healthcare Improvement)
- CBAHI (Saudi Central Board for Accreditation of Healthcare Institutions)
- GAHAR (General Authority for Healthcare Accreditation and Regulation)

113. List three benefits of effective risk management.

Answer:

- Improved patient safety

114. Reduced liability and lawsuits
115. Enhanced healthcare organization reputation
- 116. List four causes of patient safety risks.**
Answer:
 Human factors (fatigue, stress, lack of training)
 Systemic issues (poor policies or procedures)
 Environmental factors (poor lighting, noise)
 Technological failures (equipment malfunction)
- 117. List four responsibilities of a manager in risk management.**
Answer:
 Budgeting
 Planning
 Communication skills
 Staff development
 Staff appraisal
 Knowledge of rules and regulations
- 118. List four strategies for managing risk and error in healthcare settings.**
Answer:
 Identify potential risks through assessments
 Develop policies and procedures
 Provide staff training and education
 Encourage reporting of near misses and adverse events
 Conduct root cause analysis
 Implement corrective actions
- 119. List four common patient safety risks in healthcare settings.**
Answer:
 Medication errors
 Hospital-acquired infections (HAIs)

Patient falls and injuries
 Diagnostic errors
 Pressure ulcers

- 120. List four steps of Root Cause Analysis (RCA).**

Answer:

Identify the adverse event
 Gather data and information
 Analyze the data to identify causes
 Implement corrective actions

- 121. List four examples of lower-level managers.**

Answer:

Supervisors
 Foremen
 Section officers
 Superintendents

- 122. List four challenges facing quality and safety in healthcare.**

Answer:

Complexity of healthcare systems
 Limited resources
 Rapidly changing healthcare environment
 Growing focus on value-based care

- 123. List four key quality and safety concepts.**

Answer:

Patient centered care
 Evidence based practice
 Continuous quality improvement
 Safety culture
 Just culture

Leadership

- 124. What is leadership?**

- a. Ignoring people
- B) Influencing and guiding others
- C) Sleeping at work
- D) Avoiding responsibility

Answer: B

- 125. Leadership is the art of:**

- a. Fighting people
- B) Motivating people to achieve a goal

- C) Playing games
- D) Giving orders only

Answer: B

126. A good leader is a:

- a. Poor communicator
- B) Silent person
- C) Good communicator
- D) Angry person

Answer: C

127. Leaders know when to:

- a. Sleep
- B) Talk and listen
- C) Shout
- D) Leave

Answer: B

128. Leaders are:

- a. Irresponsible
- B) Accountable
- C) Lazy
- D) Careless

Answer: B

129. Leaders think about:

- a. Only today
- B) The past only
- C) Long-term goals
- D) Nothing

Answer: C

130. A leader is:

- a. Self-motivated
- B) Easily tired
- C) Always sad
- D) Not active

Answer: A

131. Good leaders are:

- a. Confused
- B) Confident
- C) Weak
- D) Fearful

Answer: B

132. Leaders care about:

- a. Money only
- B) Themselves only
- b. People and team
- D) No one

Answer: C

133. Leaders can handle:

- a. Only happiness
- B) Stress and frustration
- C) Nothing
- D) Anger only

Answer: B

134. A manager becomes a manager by:

- a. Personal qualities
- B) Position
- C) Luck
- D) Friends

Answer: B

135. A leader becomes a leader by:

- a. Position
- B) Salary
- C) Personal qualities
- D) Age

Answer: C

136. A manager has:

- a. No rights
- B) Formal rights
- C) Personal rights only
- D) No power

Answer: B

137. A leader has:

- a. Formal rights
- b. B) Legal rights
- C. No formal rights
- D) Written rights

Answer: C

138. People follow managers because of:

- a. Love
- B) Fear
- C) Job description
- D) Friendship

Answer: C

139. People follow leaders:

- a. By force
- B) Voluntarily
- C) By law
- D) By order

Answer: B

140. A manager is responsible for:

- a. Nothing
- B) Only self
- C) Self and subordinates
- D) Only leader

Answer: C

141. A leader's main role is to:

- a. Punish people
- B) Influence people
- C) Ignore people
- D) Control money

Answer: B

142. Management is:

- a. Temporary
- B) Unstable
- C) Stable
- D) Weak

Answer: C

143. Leadership is:

- a. Permanent
- B) Temporary
- C) Fixed
- D) Stable

Answer: B

144. A manager focuses on:

- a. Personal goals
- B) Organizational goals
- C) No goals
- D) Fun

Answer: B

145. A leader focuses on:

- a. Money only
- B) Group goals and satisfaction
- C) Personal gain
- D) No goals

Answer: B

146. Good leaders use:

- a. Reactive plans
- B) Proactive plans
- C) No plans
- D) Old plans

Answer: B

147. Leaders help define:

- a. Games
- B) Vision and mission
- C) Holidays
- D) Salaries

Answer: B

148. Leaders develop:

- a. Risk management plans
- B) Food plans
- C) Travel plans
- D) Fun plans

Answer: A

149. Leaders involve:

- a. No one
- B) Only managers
- C) Stakeholders
- D) Only leaders

Answer: C

150. Stakeholders include:

- a. Patients and staff
- B) Only doctors
- C) Only managers
- D) Only families

Answer: A

151. Leaders create a culture of:

- a. Fear
- B) Safety and quality
- C) Silence
- D) Punishment

Answer: B

152. Leaders establish:

- a. Sports teams
- B) Quality committee
- C) Music groups
- D) Clubs

Answer: B

153. Leaders define:

- a. No goals
- B) Quality and safety goals
- C) Personal goals only
- D) Random goals

Answer: B

154. Leaders provide:

- a. No resources
- B) Enough resources

- C) Less resources
- D) Old resources

Answer: B

155. Leaders:

- a. Ignore progress
- B) Monitor and evaluate progress
- C) Stop work
- D) Avoid change

Answer: B

156. A quality and safety framework is:

- a. A plan
- B) A game
- C) A tool only
- D) A rule only

Answer: A

157. Stakeholders include:

- a. Patients and staff
- B) Only managers
- C) Only leaders
- D) No one

Answer: A

158. Transparency means:

- a. Hiding errors
- B) Open communication
- C) Ignoring problems
- D) Silence

Answer: B

159. Education and training help staff to:

- a. Sleep
- B) Gain knowledge and skills
- C) Avoid work
- D) Play

Answer: B

160. Leaders monitor progress using:

- a. Games
- B) Data and metrics
- C) Stories
- D) Guessing

Answer: B

161. Reducing hospital infections is an example of:

- a. Playing
- B) Quality improvement
- C) Resting
- D) Ignoring

Answer: B

162. A medication error is a:

- a. Success
- B) Patient safety incident
- C) Plan
- D) Goal

Answer: B

163. Limited resources mean:

- a. Many resources
- B) No resources
- C) Few resources
- D) Extra resources

Answer: C

164. Resistance to change means:

- a. Accepting change
- B) Refusing change
- C) Creating change
- D) Loving change

Answer: B

165. Sustainability means:

- a. Short-term success
- B) Long-term continuation
- C) Stopping work
- D) Quick change

Answer: B

166. Leaders should:

- a. Ignore safety
- B) Focus on quality and safety
- C) Avoid plans
- D) Work alone

Answer: B

167. A just culture means:

- a. Blaming people
- B) Fair treatment
- C) Punishment only
- D) Ignoring errors

Answer: B

Communication

168. Effective communication helps to:

- a. Increase errors
 - B) Reduce medical errors
 - C) Ignore patients
 - D) Delay care
- Answer: B**
- 169. Clear communication prevents:**
- a. Success
 - B) Understanding
 - C) Mistakes
 - D) Learning
- Answer: C**
- 170. Good communication improves:**
- a. Patient outcomes
 - B) Games
 - C) Noise
 - D) Confusion
- Answer: A**
- 171. Effective communication helps patients feel:**
- a. Confused
 - B) Ignored
 - C) Satisfied
 - D) Angry
- Answer: C**
- 172. Communication supports:**
- a. Fighting
 - B) Teamwork
 - C) Silence
 - D) Isolation
- Answer: B**
- 173. Good communication helps in:**
- a. Delaying care
 - B) Giving proper care
 - C) Ignoring care
 - D) Stopping care
- Answer: B**
- 174. Communication helps healthcare workers to:**
- a. Work alone
 - B) Collaborate
 - C) Stop working
 - D) Argue
- Answer: B**

- 175. Effective communication creates a culture of:**
- a. Fear
 - B) Safety
 - C) Silence
 - D) Confusion
- Answer: B**
- 176. Patient satisfaction increases with:**
- a. Poor communication
 - B) No communication
 - C) Good communication
 - D) Late communication
- Answer: C**
- 177. Communication helps patients feel:**
- a. Uninformed
 - B) Empowered
 - C) Lost
 - D) Weak
- Answer: B**
- 178. Open communication helps in:**
- a. Hiding problems
 - B) Early detection of issues
 - C) Ignoring risks
 - D) Delaying reports
- Answer: B**
- 179. Early reporting helps to:**
- a. Increase problems
 - B) Prevent major problems
 - C) Stop work
 - D) Create errors
- Answer: B**
- 180. Communication helps to:**
- a. Increase errors
 - B) Prevent errors
 - C) Ignore mistakes
 - D) Delay care
- Answer: B**
- 181. Miscommunication can lead to:**
- a. Safety
 - B) Success
 - C) Serious consequences
 - D) Happiness

Answer: C

182. SBAR is used to:

- a. Confuse staff
- B) Improve communication
- C) Delay messages
- D) Stop reporting

Answer: B

183. Transparency helps to:

- a. Hide errors
- B) Build accountability
- C) Create fear
- D) Stop communication

Answer: B

184. Staff should feel:

- a. Afraid to report
- B) Comfortable reporting errors
- C) Silent
- D) Ignored

Answer: B

185. Sharing information helps:

- a. No one
- B) Only leaders
- C) All healthcare staff
- D) Only patients

Answer: C

186. Communication should be:

- a. Late
- B) Clear and timely
- C) Confusing
- D) Hidden

Answer: B

187. Good communication improves:

- a. Risk
- B) Patient safety
- C) Errors
- D) Delay

Answer: B

188. Patients should be:

- a. Ignored
- B) Involved and informed
- C) Silent
- D) Afraid

Answer: B

189. Patients are encouraged to:

- a. Stay quiet
- B) Ask questions
- C) Leave care
- D) Ignore problems

Answer: B

190. Risk management requires:

- a. Only nurses
- B) Multidisciplinary teams
- C) Only administrators
- D) Only doctors

Answer: B

191. Multidisciplinary teams include:

- a. Only patients
- B) Physicians, nurses, pharmacists, and staff
- C) Only managers
- D) Only leaders

Answer: B

192. Collaboration brings:

- a. Confusion
- B) Diverse perspectives and expertise
- C) Mistakes
- D) Delay

Answer: B

193. Shared decision-making ensures:

- a. Only leaders decide
- B) All stakeholders have a voice
- C) No one decides
- D) Managers decide alone

Answer: B

194. Shared decision-making increases:

- a. Ownership and successful implementation
- B) Errors
- C) Conflicts
- D) Delay

Answer: A

195. Problem-solving in teams helps to:

- a. Identify and solve complex problems
- B) Ignore problems
- C) Create new risks
- D) Delay care

Answer: A

196. Continuous quality improvement requires:

- a. Ignoring data
- B) Collaboration
- C) Working alone
- D) Hiding mistakes

Answer: B

197. Sharing best practices and lessons learned helps to:

- a. Reduce risks
- B) Increase errors
- C) Delay care
- D) Create confusion

Answer: A

198. System-wide improvements can be achieved by:

- a. Collaboration across departments and facilities
- B) Working alone
- C) Ignoring other departments
- D) Avoiding communication

Answer: A

199. Quality improvement teams:

- a. Meet regularly to review patient outcomes
- B) Work without data
- C) Ignore problems
- D) Only focus on managers

Answer: A

200. Clear communication should:

- a. Use jargon
- B) Be concise and simple
- C) Be confusing
- D) Be ignored

Answer: B

201. Active listening means:

- a. Ignoring others
- B) Paying attention and confirming understanding
- C) Talking only
- D) Avoiding questions

Answer: B

202. Standardizing communication protocols helps to:

- a. Create confusion
- B) Ensure consistency
- C) Delay care
- D) Avoid teamwork

Answer: B

203. Open communication encourages:

- a. Fear among staff
- B) Team members to share concerns
- C) Silence
- D) Mistakes

Answer: B

204. Technology can help communication by:

- a. Using secure messaging and video conferencing
- B) Ignoring messages
- C) Stopping calls
- D) Avoiding electronic records

Answer: A

205. During critical situations, communication should be:

- a. Long and unclear
- B) Clear and concise
- C) Ignored
- D) Confusing

Answer: B

206. Regular safety meetings help to:

- a. Share incidents and risks
- B) Ignore problems
- C) Delay care
- D) Avoid teamwork

Answer: A

207. Standardized handoff protocols reduce:

- a. Confusion and risk
- B) Patient safety
- C) Collaboration
- D) Care

Answer: A

208. Incident reporting systems allow:

- a. Staff to report errors anonymously
- B) Leaders to hide errors
- C) Ignoring mistakes
- D) Delaying care

Answer: A

209. Simulation training helps staff to:

- a. Practice communication safely
- B) Avoid teamwork
- C) Ignore mistakes
- D) Work alone

Answer: A

210. Patient education improves:

- a. Understanding and safety
- B) Confusion
- C) Mistakes
- D) Errors

Answer: A

211. Language barriers occur due to:

- a. Same language
- B) Differences in language or dialect
- C) Good communication
- D) Clear instructions

Answer: B

212. Cultural barriers happen because of:

- a. Same culture
- B) Different cultural backgrounds
- C) Teamwork
- D) Open communication

Answer: B

213. Hierarchical barriers occur when:

- a. Everyone feels equal
- B) Power imbalances intimidate team members
- C) Team members speak freely
- D) Leaders listen

Answer: B

214. Technological barriers occur due to:

- a. Well-functioning tools
- B) Poor or malfunctioning communication tools
- C) Clear messages
- D) Open meetings

Answer: B

215. To overcome language barriers, we can:

- a. Ignore staff
- B) Use interpretation services
- C) Speak louder
- D) Avoid communication

Answer: B

216. To overcome cultural barriers, we can:

- a. Provide cultural competency training
- B) Ignore differences
- C) Enforce one culture
- D) Avoid teamwork

Answer: A

217. Flattening hierarchies helps to:

- a. Encourage open communication
- B) Stop staff from speaking
- C) Increase fear
- D) Ignore ideas

Answer: A

218. Regular technology maintenance ensures:

- a. Tools work properly
- B) Tools break down

- C) Errors increase
- D) Confusion

Answer: A

219. Effective communication is blocked by:

- a. Clear messages
- B) Language, cultural, hierarchical, and technological barriers
- C) Active listening
- D) Teamwork

Answer: B

220. Empowering team members helps to overcome:

- a. Language barriers
- B) Hierarchical barriers
- C) Cultural barriers
- D) Technology barriers

Answer: B

221. A common challenge to communication is:

- a. Plenty of time
- B) Time constraints
- C) Clear messages
- D) Teamwork

Answer: B

222. Information overload means:

- a. Too little information
- B) Too much information to handle
- C) Clear communication
- D) Teamwork

Answer: B

223. Interdisciplinary communication problems happen between:

- a. Same departments
- B) Different healthcare disciplines
- C) Patients only
- D) Managers only

Answer: B

224. Poor communication tools can create:

- a. Technological barriers
- B) Clear messages
- C) Teamwork
- D) Safety

Answer: A

225. To overcome time constraints, teams should:

- a. Ignore communication
- B) Prioritize communication
- C) Delay messages
- D) Avoid meetings

Answer: B

226. Clear and concise language helps to:

- a. Confuse team members
- B) Avoid misunderstandings
- C) Create errors
- D) Delay care

Answer: B

227. Standardized communication protocols help to:

- a. Reduce errors
- B) Increase confusion
- C) Hide information
- D) Delay care

Answer: A

228. Regular training and feedback help teams to:

- a. Learn effective communication
- B) Avoid teamwork
- C) Ignore errors
- D) Work alone

Answer: A

229. Implementing communication strategies improves:

- a. Errors
- B) Patient safety and outcomes
- C) Confusion
- D) Delay

Answer: B

230. Effective communication in healthcare:

- a. Can be ignored
- B) Improves patient satisfaction
- C) Creates mistakes
- D) Confuses staff

Answer: B

Evidence based practice for effective risk Management and Quality.

231. What is Evidence-Based Practice (EBP)?

- A. A random way of treating patients
- B. A systematic approach using best evidence, clinical experience, and patient preferences
- C. A method based only on tradition
- D. A non-scientific approach

Answer: B

232. EBP combines which three elements?

- A. Research, finance, and policy
- B. Doctors, nurses, and patients
- C. Evidence, clinical experience, and patient preferences
- D. Books, journals, and media

Answer: C

233. What is the main goal of EBP?

- A. Increase costs
- B. Reduce number of patients
- C. Focus only on research
- D. Improve quality of clinical care

Answer: D

234. EBP helps to improve:

- A. Patient outcomes
- B. Hospital buildings
- C. Staff salaries
- D. Medical equipment only

Answer: A

235. EBP is used to:

- A. Avoid decision-making
- B. Replace clinical judgment

- C. Ignore new research
- D. Review, analyze, and translate scientific evidence

Answer: D

236. Which type of care does EBP aim to provide?

- A. Expensive care
- B. Inefficient care
- C. Effective and efficient care
- D. Random care

Answer: C

237. EBP helps to reduce:

- A. Patient safety
- B. Number of treatments
- C. Variation in care
- D. Clinical knowledge

Answer: C

238. EBP focuses on selecting interventions that:

- A. Have the greatest chance of success
- B. Are cheapest only
- C. Are easiest to perform
- D. Require no evidence

Answer: A

239. How many goals does EBP have (according to the text)?

- A. One
- B. Two
- C. Three
- D. Five

Answer: A

240. EBP is considered a:

- A. Random method
- B. Traditional belief
- C. Scientific process
- D. Personal opinion

Answer: C

241. What are the two fundamental principles of EBP?

- A. Cost and time
- B. Research and policy
- C. Hierarchy of evidence and insufficiency of evidence alone
- D. Patient and doctor opinion

Answer: C

242. **The hierarchy of evidence means:**

- A. All evidence is equal
- B. Some evidence is stronger than others
- C. Only patient opinion matters
- D. Only books are used

Answer: B

243. **Insufficiency of evidence alone means:**

- A. Evidence is always enough
- B. Only research is needed
- C. Decisions should ignore patients
- D. Evidence alone is not enough for decision-making

Answer: D

244. **How many components does EBP have?**

- A. Two
- B. Three
- C. Four
- D. Five

Answer: B

245. **Which of the following is NOT one of the triple Es?**

- A. Evidence
- B. Expertise
- C. Expectations of patients
- D. Evaluation

Answer: D

246. **The three components of EBP are:**

- A. Evidence, expertise, and patient expectations
- B. Evidence, economy, and environment
- C. Expertise, education, and ethics
- D. Patients, nurses, and doctors

Answer: A

247. **How many steps are there in EBP?**

- A. Three
- B. Four
- C. Five
- D. Six

Answer: C

248. **Which of the following is the first step of EBP?**

- A. Apply
- B. Assess
- C. Acquire
- D. Ask

Answer: D

249. **Which step involves collecting evidence?**

- A. Acquire
- B. Ask
- C. Apply
- D. Assess

Answer: A

250. **The final step of EBP is:**

- A. Ask
- B. Acquire
- C. Apply
- D. Assess

Answer: D

251. **How many components are there in Evidence-Based Practice (EBP)?**

- A. Two
- B. Three
- C. Four
- D. Five

Answer: B

252. **What does "best evidence" mean in EBP?**

- A. Personal opinions
- B. Hospital rules
- C. Information that supports a conclusion about care effectiveness
- D. Patient emotions

Answer: C

253. **In healthcare, best evidence usually refers to:**

- A. Doctor's salary
- B. Effectiveness of interventions
- C. Hospital design
- D. Number of patients

Answer: B

254. **Clinical experience helps the practitioner to:**

- A. Ignore patient values
 - B. Make random decisions
 - C. Identify patient's unique health condition and needs
 - D. Avoid diagnosis
 - Answer: C**
255. **Clinical experience includes understanding:**
- A. Only hospital policies
 - B. Risks and benefits of interventions
 - C. Only patient age
 - D. Financial costs only
 - Answer: B**
256. **Patient preferences refer to:**
- A. Doctor's decisions only
 - B. Hospital management rules
 - C. General treatment plans
 - D. Patient's values, concerns, and expectations
 - Answer: D**
257. **Why are patient preferences important in EBP?**
- A. They replace evidence
 - B. They help in making patient-centered decisions
 - C. They reduce hospital costs only
 - D. They are not important
 - Answer: B**
258. **The three components of EBP must be:**
- A. Ignored
 - B. Used separately
 - C. Replaced by research
 - D. Integrated together
 - Answer: D**
259. **Which of the following is NOT a component of EBP?**
- A. Best evidence
 - B. Clinical experience
 - C. Patient preferences
 - D. Hospital budget
 - Answer: D**
260. **EBP process (5As) refers to:**
- A. Five components
 - B. Five steps in applying EBP
 - C. Five types of patients
 - D. Five hospital rules
 - Answer: B**
261. **One benefit of EBP for patients is:**
- A. Increased cost of care
 - B. Reduced understanding of treatment
 - C. Improved care and reduced inappropriate treatments
 - D. Less accountability
 - Answer: C**
262. **EBP helps patients by:**
- A. Decreasing consistency in care
 - B. Enhancing consistency in care
 - C. Ignoring treatment plans
 - D. Reducing safety
 - Answer: B**
263. **For nurses, EBP helps to:**
- A. Avoid decision-making
 - B. Reduce confidence
 - C. Ignore evidence
 - D. Increase confidence in practice
 - Answer: D**
264. **One benefit of EBP for healthcare organizations is:**
- A. Increased variation in services
 - B. Reduced accountability
 - C. Promotes cost-effectiveness
 - D. Decreased decision-making
 - Answer: C**
265. **The first step of EBP (Ask) involves:**
- A. Applying evidence
 - B. Developing a researchable question
 - C. Evaluating results
 - D. Ignoring patient problems
 - Answer: B**
266. **The PICO model stands for:**
- A. Patient, Input, Care, Output
 - B. Plan, Intervention, Control, Outcome

C. Population, Intervention,
Comparison, Outcome
D. Problem, Idea, Control,
Observation

Answer: C

267. **In PICO, the letter “P”
refers to:**

- A. Plan
- B. Patient/Population
- C. Practice
- D. Problem only

Answer: B

268. **The “Acquire” step means:**

- A. Applying treatment
- B. Asking questions
- C. Evaluating outcomes
- D. Searching for evidence

Answer: D

269. **Which of the following is
a source of evidence?**

- A. Ignoring research
- B. Clinical guidelines
- C. Guessing treatment
- D. Personal opinions only

Answer: B

270. **Evidence from patients
includes:**

- A. Only laboratory results
- B. Only doctor’s opinion
- C. Patient satisfaction and
experiences
- D. Hospital rules

Answer: C

271. **The purpose of the
“Appraise” step is to:**

- A. Apply treatment
- B. Search for evidence
- C. Assess the strength and quality
of evidence
- D. Ignore research findings

Answer: C

272. **Critical appraisal helps to
ensure that evidence is:**

- A. Expensive
- B. Valid, relevant, and applicable

- C. Easy to read
- D. Old

Answer: B

273. **Validity in research
means:**

- A. The study is long
- B. The evidence is telling the truth
- C. The results are complicated
- D. The study is published

Answer: B

274. **Applicability of evidence
refers to:**

- A. Cost of treatment
- B. Study design
- C. Ability to apply results to the
local population
- D. Number of patients

Answer: C

275. **The “Apply” step involves:**

- A. Ignoring patient values
- B. Choosing interventions and
integrating evidence with clinical
expertise
- C. Only reading research
- D. Writing reports

Answer: B

276. **Which of the following
helps in applying evidence?**

- A. Random guessing
- B. Guidelines, pathways, and
protocols
- C. Patient complaints only
- D. Hospital finance

Answer: B

277. **A protocol is defined as:**

- A. A patient opinion
- B. A research article
- C. Instructions for practitioners to
safeguard patients
- D. A financial plan

Answer: C

278. **The “Assess” step focuses
on:**

- A. Asking questions
- B. Searching for evidence
- C. Evaluating outcomes of practice

changes
D. Ignoring results

Answer: C

279. **Which of the following is evaluated in the Assess step?**

- A. Staff uniforms
- B. Building design
- C. Patient outcomes and satisfaction
- D. Hospital location

Answer: C

280. **Audit and feedback are used to:**

- A. Increase variation in care
- B. Identify gaps between actual and desired results
- C. Avoid evaluation
- D. Replace guidelines

Answer: B

281. **One common challenge in applying EBP is:**

- A. Too much time available
- B. Limited access to guidelines and research
- C. Excess resources
- D. Too many staff

Answer: B

282. **Healthcare providers may resist EBP because of:**

- A. High motivation
- B. Easy implementation
- C. Resistance to change
- D. Too much training

Answer: C

283. **Patient factors affecting EBP include:**

- A. Only hospital rules
- B. Only doctor decisions
- C. Building design
- D. Patient preferences and values

Answer: D

284. **Lack of time and resources leads to:**

- A. Easy application of EBP
- B. Faster decision-making

- C. Difficulty implementing EBP
- D. Better outcomes

Answer: C

285. **One strategy to overcome challenges is:**

- A. Ignoring guidelines
- B. Reducing staff
- C. Providing education and training
- D. Avoiding research

Answer: C

286. **Clinical decision support tools are used to:**

- A. Replace doctors
- B. Guide clinical decision-making
- C. Increase costs
- D. Reduce knowledge

Answer: B

287. **Policies and procedures help to:**

- A. Block EBP
- B. Support evidence-based practices
- C. Increase resistance
- D. Reduce collaboration

Answer: B

288. **Interdisciplinary collaboration means:**

- A. Working alone
- B. Ignoring other professionals
- C. Collaboration among healthcare professionals
- D. Avoiding teamwork

Answer: C

289. **Education and training in EBP help to:**

- A. Increase resistance
- B. Reduce knowledge
- C. Confuse staff
- D. Improve understanding of EBP

Answer: D

290. **A major barrier to EBP implementation is:**

- A. Too much evidence
- B. Easy access to resources

- C. Lack of challenges
- D. Insufficient time

Answer: D

List questions

291. List Strategies to Overcome challenges in applying EBP.

Provide education and training:
Educate healthcare providers on evidence-based practices and guidelines

Develop clinical decision support tools: Create tools to guide clinical decision-making

Implement policies and procedures: Develop policies and procedures to support evidence-based practices

Encourage interdisciplinary

collaboration: Foster collaboration among healthcare professionals to promote evidence-based practices

292. List the three components of EBP (Triple Es).

Answer:

Best evidence

Clinical expertise (experience)

Patient expectations/preferences

293. List the two fundamental principles of EBP.

Hierarchy of evidence

Insufficiency of evidence alone in decision-making

294. List the five steps of Evidence-Based Practice (5As).

Answer:

Ask

Acquire

Appraise

Apply

Assess

295. List four benefits of EBP for patients.

Answer:

Improved care

Reduced inappropriate treatments

Enhanced consistency in care

Increased understanding of treatment

296. List four benefits of EBP for nurses/clinical practitioners.

Answer:

Improves quality of care

Helps change practice when needed

Increases confidence

Involves them in decision-making

297. List four benefits of EBP for healthcare organizations.

Answer:

Consistent decision-making

Reduced variation in services

Cost-effectiveness

Increased accountability

298. List the four elements of the PICO model.

Answer:

Population

Intervention

Comparison

Outcome

299. List three criteria used in critical appraisal of evidence.

Answer:

Validity

Relevance

Applicability

300. List four common challenges in applying EBP.

Answer:

Limited access to evidence

Lack of time/resources

Resistance to change

Patient-related factors

Nursing Informatics for Quality Improvement:

301. **Health informatics is a:**

- A. Surgical procedure
- B. Single medical specialty only
- C. Multidisciplinary field
- D. Drug therapy method

Answer: C

302. **Health informatics mainly focuses on:**

- A. Hospital construction
- B. Use of information technology in healthcare
- C. Drug manufacturing
- D. Surgery techniques

Answer: B

303. **Nursing informatics is the application of:**

- A. Physical therapy only
- B. Herbal medicine
- C. Computer systems and software in nursing
- D. Surgery tools

Answer: C

304. **One main goal of nursing informatics is to:**

- A. Reduce patient communication
- B. Increase paperwork
- C. Avoid technology use
- D. Improve patient care and decision-making

Answer: D

305. **Nursing informatics helps in:**

- A. Avoiding documentation
- B. Managing and processing health information
- C. Destroying health records
- D. Limiting communication

Answer: B

306. **One advantage of nursing informatics is:**

- A. Increased manual work
- B. Increased paper storage
- C. Reduced patient safety
- D. Decreased time spent on documentation

D. Decreased time spent on documentation

Answer: D

307. **Nursing informatics improves relationships between:**

- A. Physicians, peers, patients, families, and teams
- B. Only doctors
- C. Only nurses
- D. Healthcare providers and patients only

Answer: A

308. **One disadvantage of nursing informatics is:**

- A. Computer malfunction
- B. Faster care
- C. Improved communication
- D. Better documentation

Answer: A

309. **A concern in nursing informatics is:**

- A. Privacy and security of health information
- B. No data collection
- C. More paper use
- D. Less technology

Answer: A

310. **Cost in nursing informatics refers to:**

- A. No training needed
- B. Hardware, software, and training expenses
- C. Free systems
- D. No maintenance required

Answer: B

One role of the informatics nurse is to:

- A. Reduce patient care quality
- B. Improve nursing practice and patient care
- C. Eliminate technology use
- D. Increase paperwork

Answer: B

311. **Informatics nurses help in:**

- A. Avoiding research
- B. Data collection for research
- C. Destroying health data
- D. Reducing documentation

Answer: B

312. **One role of informatics nurses is to support:**

- A. Financial loss
- B. Decision-making based on assessments
- C. Paper storage systems
- D. Manual errors

Answer: B

313. **Informatics nurses support:**

- A. Cost increase only
- B. Cost saving and productivity goals
- C. Reduction in patient care
- D. Avoiding technology

Answer: B

314. **Creating new tools for patient care is based on:**

- A. Traditional methods only
- B. Old paper systems
- C. New technologies
- D. Guesswork

Answer: C

315. **Integrating systems means:**

- A. Separating departments
- B. Combining different systems in healthcare
- C. Removing technology
- D. Reducing communication

Answer: B

316. **One barrier to nursing informatics is:**

- A. Unlimited budget
- B. Limited financial resources
- C. Too much training
- D. Perfect systems

Answer: B

317. **Resistance to change in nursing informatics is often due to:**

- A. Lack of patients
- B. Nurses preferring paper-based documentation
- C. Too many computers
- D. No workload

Answer: B

318. **One need in nursing informatics implementation is:**

- A. Less education
- B. No training
- C. Training and education for nurses
- D. Removing technology

Answer: C

319. **A common fear among nurses using informatics is:**

- A. Fear of computerization and making mistakes
- B. Fear of patients
- C. Fear of documentation
- D. Fear of teamwork

Answer: A

320. **EMR stands for:**

- A. Electronic Medical Record
- B. Electronic Management Report
- C. External Medical Record
- D. Electronic Medication Record

Answer: A

321. **An EMR is mainly used by:**

- A. Patients at home only
- B. Health care providers
- C. Insurance companies only
- D. Hospital visitors

Answer: B

322. **EMR includes information such as:**

- A. Only billing details
- B. Patient health history and treatment information
- C. Only doctor names
- D. Hospital equipment lists

Answer: B

323. **EHR stands for:**

- A. Electronic Health Record
- B. External Health Report

- C. Electronic Hospital Report
- D. Emergency Health Record

Answer: A

324. **A key feature of EHR is that it can be accessed:**

- A. Only in one hospital
- B. Only by doctors
- C. From several locations by different providers
- D. Only offline

Answer: C

325. **EHR helps clinicians by providing:**

- A. Random guesses
- B. Alerts and reminders for patient safety
- C. Only financial data
- D. Paper-based records

Answer: B

326. **One benefit of EHR is:**

- A. Increased medical errors
- B. Reduced data sharing
- C. Facilitates data collection for research
- D. No patient records stored

Answer: C

327. **Clinical Decision Support Systems (CDSS) stands for:**

- A. Clinical Data Support System
- B. Clinical Decision Support System
- C. Care Data Storage System
- D. Clinical Documentation Support Service

Answer: B

328. **CDSS helps nurses by:**

- A. Ignoring patient data
- B. Making random decisions
- C. Providing evidence-based recommendations
- D. Increasing paperwork

Answer: C

329. **One benefit of Clinical Decision Support Systems (CDSS) is:**

- A. Increased medical errors

- B. Improved patient outcomes
- C. Reduced decision-making
- D. Slower treatment plans

Answer: B

330. **Telehealth and telemedicine mainly use technology to:**

- A. Replace hospitals completely
- B. Deliver healthcare services remotely
- C. Increase paperwork
- D. Reduce patient care

Answer: B

331. **One benefit of telehealth is:**

- A. Limited access to care
- B. Reduced communication
- C. Improved hospital waiting times only
- D. Increased access to care for rural populations

Answer: D

332. **Coaching in nursing informatics involves using:**

- A. Robots only
- B. Paper records
- C. Artificial Intelligence (AI) for training staff
- D. Manual charts

Answer: C

333. **Wearable technology is used to:**

- A. Store hospital furniture
- B. Track health metrics like heart rate and blood pressure
- C. Replace doctors
- D. Print medical records

Answer: B

334. **One advantage of wearable technology is:**

- A. No patient monitoring
- B. Promotes preventive care and self-management
- C. Increases hospital stay
- D. Reduces health awareness

Answer: B

335. **Big Data Analytics in healthcare is used to:**

- A. Delete patient records
- B. Ignore health trends
- C. Analyze large datasets for patterns and risks
- D. Reduce decision-making

Answer: C

336. **One benefit of Big Data Analytics is:**

- A. Increasing errors
- B. Poor resource use
- C. Predicting health risks and improving care
- D. Reducing healthcare quality

Answer: C

337. **Robots in healthcare can be used for:**

- A. Only documentation
- B. Surgical assistance and supply delivery
- C. Only patient interviews
- D. Paper storage

Answer: B

338. **One type of medical robot is:**

- A. Financial robot
- B. Surgical-assistance robot
- C. Cleaning paper robot
- D. Manual robot system

Answer: B

339. **Medical robots help healthcare systems by:**

- A. Increasing workload
- B. Slowing down care
- C. Streamlining supply delivery and disinfection
- D. Eliminating technology use

Answer: C

340. **The main purpose of using robotics in healthcare is to:**

- A. Increase paperwork
- B. Improve patient care and safety
- C. Replace all nurses
- D. Reduce hospital services

Answer: B

341. **Autonomous Mobile Robots (AMRs) are mainly used to:**

- A. Perform surgeries only
- B. Transport supplies and support hospital workflows
- C. Replace doctors
- D. Store patient data

Answer: B

342. **One benefit of AMRs is:**

- A. Increased infection risk
- B. Reduced workload for nurses and caregivers
- C. Slower hospital processes
- D. Reduced patient care quality

Answer: B

343. **AMRs help hospitals by:**

- A. Increasing staffing shortages
- B. Making processes inconsistent
- C. Ensuring supplies and equipment are available where needed
- D. Removing technology use

Answer: C

344. **Cleaning and disinfection AMRs mainly help to:**

- A. Increase hospital infections
- B. Slow down patient admission
- C. Limit documentation
- D. Reduce hospital-acquired infections (HAIs)

Answer: D

345. **Surgical-assistance robots help surgeons by improving:**

- A. Paperwork accuracy
- B. Speed and accuracy in complex surgeries
- C. Hospital budgeting
- D. Patient admissions only

Answer: B

346. **Some surgical robots can:**

- A. Replace hospital buildings
- B. Work without electricity
- C. Complete tasks autonomously

under supervision
D. Only clean rooms

Answer: C

347. **Simulation platforms in surgical robotics are used for:**
A. Financial management
B. Training surgeons using AI and virtual reality
C. Patient billing
D. Medication storage

Answer: B

348. **Modular robots in healthcare are mainly used for:**
A. Only documentation
B. Enhancing other systems and performing multiple functions
C. Replacing EMR systems
D. Administrative tasks only

Answer: B

349. **Therapeutic exoskeleton robots are used for:**
A. Surgery only
B. Rehabilitation after stroke or paralysis
C. Hospital cleaning
D. Data analysis

Answer: B

350. **Autonomous Mobile Robots (AMRs) can assist healthcare staff:**
A. Only after surgery
B. Before a patient is checked in
C. Only in billing tasks
D. Only at night

Answer: B

351. **One function of AMRs in real-world healthcare is:**
A. Increasing hospital paperwork
B. Triage of patients
C. Reducing patient monitoring
D. Avoiding patient interaction

Answer: B

352. **The future of healthcare robotics will evolve with:**
A. Decline in technology
B. Machine learning and data

analytics
C. Removal of AI
D. Paper records only

Answer: B

353. **Future healthcare robots are expected to be:**
A. Slower and less accurate
B. Fully manual
C. Unable to assist humans
D. Autonomous and efficient

Answer: D

354. **Computer vision in robotics helps robots to:**
A. Ignore patient data
B. See and interpret information visually
C. Print documents
D. Store files manually

Answer: B

List questions

355. **List the definition components of nursing informatics.**
Answer:
Application of computer systems and software
Improvement of patient care
Use of technology in nursing practice
Support for clinical decision-making

356. **List the main functions of nursing informatics.**
Answer:
Managing health information
Processing patient data
Communication with healthcare providers and patients
Supporting decision-making

357. **List four advantages of nursing informatics for patients/nursing practice.**
Answer:
Improved quality of care
Decreased documentation time
Better patient safety

- Support for evidence-based care
358. **List four advantages of nursing informatics for healthcare organizations.**

Answer:

- Cost savings
- Improved productivity
- Better data management
- Enhanced communication

359. **List four ways nursing informatics improves communication.**

Answer:

- Between nurses and physicians
- Between nurses and patients
- Between healthcare teams
- Between different departments

360. **List four disadvantages of nursing informatics.**

Answer:

- Computer malfunction
- Privacy and security concerns
- High cost of systems
- Risk of inaccurate information

361. **List causes of resistance to nursing informatics.**

Answer:

- Habit of paper documentation
- Fear of technology
- Lack of training
- Difficulty adapting to change

362. **List barriers to implementing nursing informatics.**

Answer:

- Limited financial resources
- Lack of training and education
- Resistance to change
- Time management issues

363. **List the skills needed for nurses in informatics.**

Answer:

- Computer literacy
- Data management skills
- Communication skills
- Decision-making skills

364. **List the purposes of nursing informatics in healthcare.**

Answer:

- Improve patient care
- Support clinical decisions
- Enhance efficiency
- Manage health information

365. **List the uses of Autonomous Mobile Robots (AMRs) in real-world healthcare.**

Answer:

- Patient triage before check-in
- Transporting supplies and equipment
- Supporting healthcare staff in high-risk areas
- Assisting with cleaning and disinfection

366. **List the vital signs or data collected by Roomie Bot during patient triage.**

Answer:

- Temperature
- Blood oxygen level
- Medical history

367. **List the purpose of Roomie Bot in hospital settings.**

Answer:

- To assist medical staff with patient triage
- To help manage high-risk COVID-19 patients
- To collect initial patient data

368. **List the type of technology used by Roomie Bot.**

Answer:

- Intel-based technology
- Artificial intelligence (AI)
- Robotics systems

369. **List the roles of robots in patient care before hospital admission.**

Answer:

- Collecting patient data

Measuring vital signs
Supporting triage decisions

370. **List the key technologies that will shape the future of healthcare robotics.**

Answer:

Machine learning
Data analytics
Computer vision
Artificial intelligence

371. **List the expected future capabilities of healthcare robots.**

Answer:

Autonomous task performance
Improved accuracy and efficiency
Better decision-making support

372. **List three examples of tasks performed by healthcare robots in hospitals.**

Answer:

Patient triage
Supply delivery
Cleaning and disinfection

373. **List the benefits of using autonomous robots in healthcare settings.**

Answer:

Improved efficiency
Reduced workload for staff
Increased patient safety
Reduced infection risk

374. **List the functions of computer vision in healthcare robotics.**

Answer:

Helps robots interpret visual data
Supports recognition of patient conditions
Assists in surgical precision and monitoring

Continuous Quality Improvement

375. **List the three types of quality management in healthcare.**

Answer:

Quality Assurance (QA)
Quality Control (QC)
Quality Improvement (QI)

376. **List the goal of Quality Assurance (QA).**

Answer:

Prevent errors by establishing systems, protocols, and standards

377. **List the goal of Quality Control (QC).**

Answer:

Detect and fix deviations from quality standards

378. **List the goal of Quality Improvement (QI).**

Answer:

Continuously enhance care delivery and patient outcomes

379. **List the principles of Quality Improvement in healthcare.**

Answer:

Patient-centered care
Continuous improvement
Collaboration
Evidence-based practice
Transparency

380. **List the focus areas of patient-centered care.**

Answer:

Patient needs
Patient preferences
Patient values

381. **List the roles of data collection and analysis in quality improvement.**

Answer:

Identify areas for improvement
Set goals and objectives
Monitor progress
Evaluate outcomes
Inform decision-making

382. **List what data analysis helps to identify in healthcare quality improvement.**

- Answer:**
Gaps in care
- 383. List what data is used for in setting quality improvement goals.**
Answer:
Establish realistic targets
- 384. List what monitoring progress in quality improvement involves.**
Answer:
Tracking data to assess effectiveness of interventions
- 385. List the main challenges and opportunities for quality improvement.**
Answer:
Data quality and availability
Stakeholder engagement
Sustainability
Spread and scale
- 386. List what is meant by data quality and availability.**
Answer:
Ensuring accurate and reliable data
- 387. List who should be involved in stakeholder engagement.**
Answer:
Patients
Families
Healthcare professionals
- 388. List the purpose of stakeholder engagement in quality improvement.**
Answer:
Involving patients, families, and healthcare professionals in the process
- 389. List what sustainability in quality improvement means.**
Answer:
Ensuring long-term viability of quality improvement initiatives
- 390. List what is required for successful sustainability.**
Answer:
Long-term continuation of improvement initiatives
- 391. List what spread and scale refers to in quality improvement.**
Answer:
Disseminating successful initiatives across the organization
- 392. List the goal of spreading quality improvement initiatives.**
Answer:
Expanding successful practices across the organization
- 393. List one key requirement for good data in quality improvement.**
Answer:
Data must be accurate
- 394. List the main groups involved in quality improvement stakeholder engagement.**
Answer:
Patients
Families
Healthcare professionals
- MCQ:**
- 395. Quality Assurance (QA) is:**
a. Reactive process
B. Proactive process
C. Random process
D. Financial process
Answer: B
- 396. The main goal of QA is to:**
a. Detect errors after they occur
B. Increase hospital costs
C. Prevent errors before they occur
D. Reduce staff
Answer: C
- 397. Quality Control (QC) is a:**
a. Preventive process
B. Proactive process

- C. Reactive process
- D. Research process

Answer: C

398. The main goal of QC is to:

- a. Ignore standards
- B. Detect and fix problems after they occur
- C. Prevent all diseases
- D. Increase paperwork

Answer: B

399. Quality Improvement (QI) focuses on:

- a. Random care delivery
- B. Continuous improvement of care processes
- C. Only financial management
- D. Reducing healthcare services

Answer: B

400. The goal of QI is to:

- a. Reduce patient care
- B. Continuously enhance care and outcomes
- C. Stop data collection
- D. Replace healthcare staff

Answer: B

401. QA, QC, and QI together are called:

- a. Clinical governance
- B. Total Quality Management
- C. Health informatics
- D. Risk management

Answer: B

402. One principle of quality improvement is:

- a. Random practice
- B. Patient-centered care
- C. Cost reduction only
- D. Individual decision-making only

Answer: B

403. Continuous improvement means:

- a. Never changing care processes
- B. Regularly assessing and improving care processes
- C. Reducing quality checks
- D. Avoiding feedback

Answer: B

404. Evidence-based practice in quality improvement means:

- a. Using guesses in care
- B. Ignoring research
- C. Using best available research evidence
- D. Only using tradition

Answer: C

405. One role of data collection in quality improvement is to:

- A. Ignore patient outcomes
- B. Identify areas for improvement
- C. Increase paperwork
- D. Reduce care quality

Answer: B

406. Data analysis in QI helps to:

- A. Hide clinical problems
- B. Pinpoint gaps in care
- C. Reduce patient safety
- D. Avoid decision-making

Answer: B

407. Data is used in QI to set:

- A. Random treatments
- B. Financial budgets only
- C. Goals and objectives
- D. Staff uniforms

Answer: C

408. Monitoring progress in QI means:

- A. Stopping data collection
- B. Tracking data to assess interventions
- C. Ignoring results
- D. Reducing evaluation

Answer: B

409. Evaluating outcomes in QI helps to:

- A. Increase hospital errors
- B. Determine impact on patient care
- C. Reduce data use
- D. Avoid feedback

Answer: B

410. **The Donabedian model focuses on:**

- A. Cost only
- B. Structure, process, and outcomes
- C. Medication only
- D. Patient satisfaction only

Answer: B

411. **Root Cause Analysis (RCA) is used to:**

- A. Increase workload
- B. Identify underlying causes of problems
- C. Avoid problem solving
- D. Reduce documentation

Answer: B

412. **The 5 Whys technique is used to:**

- A. Increase errors
- B. Ask “why” repeatedly to find root cause
- C. Reduce analysis
- D. Avoid investigation

Answer: B

413. **The main purpose of FMEA is to:**

- A. Increase hospital errors
- B. Identify potential failures and their impact
- C. Reduce documentation
- D. Avoid data collection

Answer: B

414. **SWOT analysis identifies:**

- A. Structure, work, outcome, time
- B. Symptoms, work, outcome, treatment
- C. Strengths, weaknesses, opportunities, threats
- D. Safety, weight, oxygen, therapy

Answer: C

415. **A Pareto chart is used to:**

- A. Store patient records
- B. Identify most significant contributing factors to a problem
- C. Measure blood pressure
- D. Replace clinical decisions

Answer: B

416. **The Pareto principle is also known as:**

- A. 50/50 rule
- B. 70/30 rule
- C. 80/20 rule
- D. 90/10 rule

Answer: C

417. **One challenge in quality improvement is:**

- A. Unlimited data access
- B. Data quality and availability
- C. No need for evaluation
- D. No staff involvement

Answer: B

418. **Stakeholder engagement means involving:**

- A. Only doctors
- B. Patients, families, and healthcare professionals
- C. Only nurses
- D. Only managers

Answer: B

419. **Sustainability in quality improvement refers to:**

- A. Short-term projects only
- B. Long-term viability of initiatives
- C. Reducing quality
- D. Avoiding change

Answer: B

420. **Spread and scale in quality improvement means:**

- A. Limiting success to one ward
- B. Discarding improvements
- C. Increasing errors
- D. Disseminating successful initiatives across organization

Answer: D